	SIADH	DI
Hormone	High ADH	Low ADH
Blood (Serum) Findings	• Hyponatremia • Low osmolality • Hypervolemia	• Hypernatremia • High osmolality • Hypovolemia
Urine Findings	• High osmolality • Low urine output	• Low osmolality • High urine output (polyuria)
Other S/S	Think hyponatremia and hypervolemia!	Think hypernatremia and hypovolemia!
Causes	Ectopic (lung stuff)	• Genetic • Head injury (pituitary/secondary issue) • Nephrogenic

	Cushings	Addisons
Hormone	High cortisol (glucocorticoid)	Low cortisol (glucocorticoid)
S/S	• Moon face • Buffalo hump • Muscle wasting • Fragile skin with striae, bruising, ulcers • Osteoporosis • HTN • Hirsutism • Amenorrhea • Hyperglycemia • Emotional lability	• Hypoglycemia • Fatigue • Weight loss • Hypotension • Dehydration • Anorexia • Hyperpigmentation of skin • Increased infections • Hyponatremia • Hyperkalemia
Complications	Risk of infection	Adrenal crisis/Addisonian crisis Risk of infection Decreased stress response
Cause	• Overuse of steroids • Pituitary or adrenal adenoma → G, M, A affected	• Autoimmune • Destructive adrenal tumors • Meningitis → G, M, A affected • Destructive pituitary tumors → G, M, A affected

	Hyperthyroidism	Hypothyroidism
Hormone	High T3/T4	Low T3/T4
Example	Grave's Disease	Hashimoto's Thyroiditis
Cause	• Genetic • Autoimmune	• Genetic • Autoimmune
S/S	• Flushed warm skin • Diaphoresis • Heat intolerance • Weight loss • Exophthalmos • Goiter • Fine, brittle hair • Tachycardia • HTN • Change in bowel patterns and periods • Increased appetite • Restless • Nervous • Fatigued muscles • Tremors	• Goiter • Weight loss • Dry skin • Cold sensitivity • Hair loss • Bradycardia • Elevated cholesterol • Change in periods • Constipation • Depression • Fatigue *Myxedema - periorbital edema, puffy face, thick tongue
Complications	Thyrotoxic crisis/thyroid storm	Myxedema coma

DM1	DM2

Pathophysiology	Destruction of pancreatic beta cells → NO insulin production → hyperglycemia	Gradual inadequate secretion of insulin from pancreas AND/OR insulin resistance → hyperglycemia
S/S Onset	Quickly	Insidious
Cause/Risk Factors	• Genetic • Virus exposure • Autoimmune	• Metabolic syndrome • Increased age • Non-white • Family history • Sedentary lifestyle • Smoking • Gestational diabetes
Treatment	SQ insulin	1. Diet and exercise (lifestyle modifications) 2. PO meds 3. SQ insulin
Diet modifications	• Carb counting • Complex carbs and fiber • Good protein intake • Decreased fat intake • Nonnutritive sweeteners and decreased sugary drinks • Limit alcohol to 1-2 drinks per day and drink with food	
Specific Complication	DKA	HHS

	DKA	**HHS**
Ketones?	Yes	No
Blood Sugar Level	High (300-600)	SUPER high (600+)
Acidosis?	Yes	No
Cause/Risk Factor	• Illness/infection • Stress • Increased food/alcohol intake *DKA-specific: new DM1 diagnosis, insulin dosage error	
S/S	• Dehydration • Ketonuria • Glucosuria • Hypotension • Low electrolytes • N/V • Weakness • Sunken eyes • Acetone breath • Kussmaul respirations • Hot dry flushed skin • Dry mucous membranes • Weak pulses	• Dehydration • Glucosuria • Confusion • Drowsy • Blurry vision • Dry mouth • Weakness